

A CRASH COURSE

Corporate Wellness That Actually Works.

Chronic disease is the highest cost problem in your work family — and most of the people who have it don't know it yet. This is a short course in the one lever that moves everything else: awareness.

The country has an awareness problem, not an information problem.

Chronic conditions are diagnosable, treatable and, in several cases, reversible. The reason they keep compounding inside workforces is simpler than medicine: people are walking around with them and have never been told. Below is how many US adults with each condition are completely unaware they have it.

50%

UNAWARE

Hypertension

Nearly half of US adults. Damages kidneys and brain daily; the leading cause of dementia in the US.

90%

UNAWARE

Pre-Diabetes

42.6% of US adults. A rare two-to-three year window where, if noticed, diabetes can be avoided entirely.

25%

UNAWARE

Type 2 Diabetes

A quarter of people living with full Type 2 Diabetes, with heavy daily symptoms, do not know they have it.

90%

UNAWARE

Chronic Kidney Disease

14.6% of adults, higher than any time on record. A handful of unnoticed years escalates it to dialysis.

60%

UNAWARE

Depression

18.6% of adults, higher than any time on record. Creates toxic environments and unhappy workplaces.

1/2

Take any hundred employees on your roster. Statistically, dozens are carrying a condition that is actively costing you money, and the majority of them have no idea. You are already paying for this. The only question is whether anyone knows about it in time to change the outcome.

OKLAHOMA

The national picture is the floor. Oklahoma sits below it.

Every figure on the previous page describes the United States as a whole. Oklahoma employers are working with a population that carries these conditions at a higher rate and catches them later, which means the awareness gap in your building is wider than the national average, not narrower.

Oklahoma-specific prevalence and cost figures to be inserted here once supplied.

INDIVIDUAL LEVEL

A slow loss, absorbed quietly.

Unnoticed conditions do their damage on a schedule nobody is watching. Blood pressure erodes kidney and brain function every day it goes untreated. Pre-diabetes has a two-to-three year window before it becomes permanent. Kidney disease takes only a few unnoticed years to reach dialysis.

The person living it experiences fatigue, irritability and brain fog, and files it under getting older or a hard week. They cannot act, because nobody has given them anything to act on.

ORGANIZATIONAL LEVEL

It arrives as cost, culture and churn.

Untreated chronic disease shows up on your claims report as late-stage intervention, the most expensive possible version of every condition. It shows up on your floor as absence, presenteeism and preventable disability.

Depression is the one most often missed. Sixty percent of the people who have it don't know, and its effects are cultural before they are clinical: innovation, creativity, productivity and retention all move with it.

None of this is a medical mystery. It is a communication failure — and communication is the one part of it you can fix this year.

People fix what they know about.

Awareness is not a soft outcome. When a population is simply told what is happening inside their own bodies, in plain language, prevalence itself moves. These are the measured returns of population health awareness, delivered with only a few hours annually at the workplace.

RETURN ON INVESTMENT

\$18:1

A CDC calculated reduction in medical spend of **\$1.8 million** per 1,000 participants.

33%

reduction in cases of
Pre-diabetes

10%

reduction of Diabetes
prevalence

38%

fewer cases of
Hypertension

36%

more people with
Healthy Blood Pressure

Knowledge is the intervention

No employer can make an adult change their diet. But you can make sure that every adult on your roster knows their numbers, understands what those numbers mean, and knows what step comes next. That is the whole mechanism. Almost everything a wellness program hopes for happens downstream of it.

Self-empowerment, not surveillance

Results belong to the employee, delivered privately to their own phone. The company never sees an individual's health data. What the employee gets is agency: their results, what they mean, likely conditions, trends over time and recommended next steps.

STORIES OF HEALTH

"It's a win-win for your employees. It's a win-win for the company!"

Laurie Havron, *HR Leader*
TTCU Federal Credit Union

"Snapshot Health is a game changer for us."

David Fleet, *Executive VP of Benefits*
Summit Benefits, Inc.

"I am so proud of our patients!"

Brianna Wright, *Health Team Lead*
Snapshot Health

Make participation the easiest thing an employee does all month.

Awareness scales with participation, and participation scales with friction removed plus a reason to show up. Two mechanics do most of the work.

1 MECHANIC

A points system

Points turn health into something visible and cumulative. Screening earns points. Watching your results video earns points. Following up with a provider earns points. Bringing a spouse earns points.

The value is in the scoreboard, not the prize. A points balance gives people a running, low-stakes reason to take the next action, and gives you a clean measure of engagement that has nothing to do with anyone's private results.

2 MECHANIC

Incentives worth the five minutes

Incentives should reward the behavior you can actually ask for — showing up, pressing play, following up — never an outcome like a number on a chart. Reward the action and the outcomes follow on their own.

Tie incentives to things employees already value: premium contributions, paid time, real money, real goods. A modest incentive attached to a five-minute screening is the cheapest population health data an employer can buy.

REMOVE THE BARRIERS FIRST

No app to download. No password to memorize or store. Screening happens onsite, on the clock, in five to ten minutes per employee, spouses included. Results arrive as a short personalized video in a text thread, where people already are. Every barrier you remove is participation you get back.

Just press play.

Health is contagious. Build the conditions for it to spread.

A screening event is a moment. A culture is what makes the next one easy. Four habits compound faster than any single initiative.

Monthly prizes

Make health a recurring event rather than an annual one. A monthly drawing keeps the program in the room every four weeks, and it lets a small budget do the work of a much larger one because anticipation, not size, drives participation.

Free things for engagement

Small, immediate, no strings: lunch for the team with the highest screening rate, water bottles, walking shoes, a paid hour back. Employees read free things correctly — as the company spending its own money on them, on purpose.

Togetherness

Group activities beat individual challenges, because almost nobody sustains a health change alone and almost everybody sustains one with their shift, their department, their crew. Walking groups, step challenges by team, a standing Friday activity.

Team building should be health-focused

You are already budgeting for team building. Point it at movement, food and rest instead of a conference room, and the same spend buys morale and health at once. Nothing new to fund — just a different default.

THE POINT

Every one of these is cheap. None of them are about medicine. They exist so that when the screening comes around again, showing up is the normal thing to do.



05 WHAT ORGANIZATIONAL HEALTH LOOKS LIKE

Employees feel a greater sense that they are working for an organization that cares for them.

That sentence is the whole return on culture. It is also the hardest thing to buy and the easiest thing to prove. An employer who screens its people, tells them the truth about their health privately, and then keeps showing up every month has said something no benefits brochure can say.

Retention

People do not leave employers who are visibly invested in them.

Recruiting

A wellness program with real screening is a differentiator candidates can understand in one sentence.

Trust

Private results and no individual reporting is how a health program earns the room instead of unsettling it.

An aggregate report turns a wellness budget into a decision.

Screening produces one more asset besides healthier people: a de-identified picture of your own population. It tells you where the problems actually are in your workforce, rather than where the industry average says they should be.

1 See the obvious problem areas

Aggregate results surface the two or three conditions your population is actually carrying, and which locations, shifts or departments carry them most.

2 Implement the low-hanging fruit

Most findings have a cheap, unglamorous fix: what's in the break room, whether the night shift can reach a provider, whether anyone has told the floor what a good blood pressure is.

3 See the effect of those solutions

Next year's screening measures the same population against the same markers. You find out whether what you changed worked, which is more than most benefits spending ever tells you.

4 Track progress year over year

Tracking awareness means your wellness program can target, adapt and amplify. Year one is a baseline. Every year after that is a comparison, and the guide to getting better.

AGGREGATE REPORT

De-identified and population-level by design. Leadership sees the shape of the workforce's health; no individual's results are ever exposed. That boundary is what makes the data usable and the program trusted at the same time.

**Low cost. High impact.
Easy participation.**

A few hours onsite, once a year, is where this starts. Set up a screening at snapshot.health.

